22

22 Independent Radiology

The policy provisions for radiology providers can be found in the *Alabama Medicaid Agency Administrative Code,* Chapter 34.

22.1 Enrollment

Alabama Medicaid's Fiscal Agent enrolls Independent Radiology providers and issues provider contracts to applicants who meet the licensure and certification requirements of the state of Alabama, the Code of Federal Regulations, the *Alabama Medicaid Agency Administrative Code*, and the *Alabama Medicaid Provider Manual*.

Federal requirements mandate providers re-validate periodically with the Alabama Medicaid program. Providers will be notified when they are scheduled to re-validate. Failure to re-validate and provide appropriate documentation to complete enrollment will result in an end-date being placed on the provider file. Once a provider file has been closed for failure to timely re-validate, providers will have to submit a new application for enrollment.

Refer to Chapter 2, Becoming a Medicaid Provider, for general enrollment instructions and information. Failure to provide accurate and truthful information or intentional misrepresentation might result in action ranging from denial of application to permanent exclusion.

National Provider Identifier, Type, and Specialty

A provider who contracts with Alabama Medicaid as an independent radiology provider is added to the Medicaid system with the National Provider Identifiers provided at the time application is made. Appropriate provider specialty codes are assigned to enable the provider to submit requests and receive reimbursements for radiology-related claims.

NOTE:
The 10-digit NPI is required when filing a claim.

Independent Radiology providers are assigned a provider type of 29 (Independent Radiology). Valid specialties for Independent Radiology providers include the following:

- 292 Mammography
- 327 Nuclear Medicine Practitioner
- 570 Physiological Lab (Independent Diagnostic Testing Facility)
- 291 Portable X Ray Clinic
- 290 Free Standing X-Ray
- 021 Cardiac Electrophysiology
- 996 Nuclear Medicine in an Independent Radiology
- 995 Radiology Clinic (CRS)

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Enrollment Policy for Independent Radiology Providers

To participate in Medicaid, Independent Radiology providers must meet the following requirements:

- Possess certification as a Medicare provider
- Possess a Physician's Supervisory Certification and utilize certified technicians for ultrasounds, Doppler services, and non-invasive peripheral vascular studies
- Exist independently of any hospital, clinic, or physician's office
- Possess licensure in the state where located, when it is required by that state
- For mammography services, possess a certification issued by the FDA.

22.2 Benefits and Limitations

This section describes program-specific benefits and limitations. Refer to Chapter 3, Verifying Recipient Eligibility, for general benefit information and limitations. Refer to Chapter 7, Understanding Your Rights and Responsibilities as a Provider, for general criteria on Medical Necessity/ Medically Necessary Care

Radiology services are professional and technical radiological services, ordered and provided under the direction of a physician or other licensed practitioner of the healing arts. Within the scope of his practice as defined by state law and are provided in an office or similar facility other than an outpatient department of a hospital or clinic and meets the requirements for participation in Medicare. Radiology services are restricted to those that are described by procedures in the CPT manual. Providers will be paid only for covered services, which they actually perform.

NOTE:

PAs for radiation treatment and/ or management services must be requested within 30 days of providing the service. Refer to chapter 4 and 28 for more information.

Mammography: Screening mammogram and Diagnostic mammogram

A mammogram is a low dose X-ray of breast tissue to detect early signs of breast cancer and breast diseases in women and men. Providers performing mammograms must have the mammography contract.

Screening Mammograms

An Independent Radiology provider may perform screening mammogram, as a preventative radiological procedure for a woman without signs or symptoms of breast disease. A screening mammogram includes a physician’s interpretation of the results of the procedure. Per the recommendation of the American Cancer Society, Medicaid allows one screening every 12 months for recipients age 40 through 64.

This guideline is for recipients with average risk, meaning there is no family or personal history of breast cancer. A screening should be billed with the following procedure code:

- 77067 - screening mammography, bilateral (2-view study of each breast), including computer-aided detection (cad) when performed.

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Screening mammogram should be reported with one of the following ICD-10 diagnosis codes:

- Z12.31: Encounter for screening mammogram for malignant neoplasm of breast.
- Z12.39: Encounter for other screening for malignant neoplasm of breast.

Diagnostic Mammograms

An Independent Radiology provider may perform diagnostic mammogram for a man or woman with signs or symptoms of breast disease, personal history, or family history. Diagnostic mammograms are more detailed than a screening. Diagnostic mammograms use specialized techniques that help determine if the symptoms reveal the presence of cancer and should include a physician's interpretation of the results of the procedure. Services are unlimited and should be billed with one of the following procedures:

- 77065 - Diagnostic mammography, including CAD when performed unilateral.
- 77066 - Diagnostic mammography, including CAD when performed bilateral.

Sonography - Breast ultrasound

Breast ultrasound is used when a change has been seen on a mammogram or when a change is felt but does not show up on a mammogram. In some instances, the sonography may be completed without having a mammogram, but it still requires a physician's order. Beginning on January 1, 2015, the complete examination is reported with one of the following codes:

- 76641 - Ultrasound, breast, unilateral, real time with image documentation including axilla when performed, completely. This consists of an ultrasound examination of all four quadrants of the breast and the retroareolar region. It also includes ultrasound examination of the axilla, if performed.
- 76642 - Ultrasound, breast, unilateral, real time with image documentation, including axilla when performed, limited. This is focused exam of the breast that is limited to one or more of the elements included in 76641. (e.g., a focused examination limited to one or more elements of 76641, but not all four).

You may report either 76641 or 76642 once, per breast, per session. Both codes are unilateral: If medical necessity requires bilateral imaging, you may append modifier LT (left) and RT (right).
As with all ultrasound examinations, there must be a thorough evaluation of the anatomic area, image documentation, and a final written report to ensure that it is separately reportable. When performing a screening examination, list the screening diagnosis first, regardless of the finding or procedure perform due to the outcome of the screening. An ultrasound screening examination of the breast should be reported with the following ICD-10 diagnostic code:

- Z12.39: Encounter for other screening for malignant neoplasm of breast

An Independent Radiology provider may bill for obstetrical ultrasounds. Ultrasound payment is limited to one per day. Effective 10/1/2019, Medicaid

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no longer requires prior authorization for obstetrical ultrasounds (CPT 76801-76828). All ultrasounds must be medically necessary with medical diagnosis documented supporting the benefit of the ultrasound procedure. MCPs should refer to Chapter 24 for details regarding care coordination.

LDCT: Low Dose Computed Tomography

The 3-D image provided by the LDCT indicates if individuals are at high risk of developing lung cancer but have no signs or symptoms.

The LDCT utilizes specialized x-ray equipment combined with state-of-the-art computer systems. LDCT produces numerous cross-sectional images of sufficient quality to detect many abnormalities. This screen should be billed with the following procedure code:

71271 - CT, thorax, low dose for cancer screening without contrast material.

LDCT is recommended for individuals that meet the following criteria:

- Be 50–80 years of age;
- Be asymptomatic (no signs or symptoms of lung cancer);
- Have a tobacco smoking history of at least 20 pack-years (one pack-year = smoking one pack per day for one year);
- Be a current smoker or have quit within the last 15 years;
- Does not have a health problem that substantially limits life expectancy or the ability or willingness to have curative lung surgery;
- Has not received a LDCT in less than 12 months.

LDCT should be reported with the following ICD-10 code:

- F17.210 Nicotine dependence, cigarettes, uncomplicated (most used and accepted)
- F17.211: Nicotine dependence from cigarettes in remission
- F17.213: Nicotine dependence from cigarettes with withdrawal
- F17.218: Nicotine dependence from cigarettes with other nicotine-induced disorders
- F17.219: Nicotine dependence from cigarettes with unspecified nicotine-induced disorders
- Z87. 891 for Personal history of nicotine dependence

Nerve Conduction Studies and Electromyography

Nerve Conduction Studies (NCS) measure action potentials recorded over the nerve or from an innervated muscle. Nerve Conduction Velocity (NCV), one aspect of NCS, measured between two sites of stimulation or between a stimulus and a recording site. It is axiomatic that neurodiagnostic studies are an extension of the history and physical examination of the patient and must be performed as part of a face-to-face encounter. Obtaining and interpreting nerve conduction velocities requires extensive interaction between the performing physician and patient and is most effective when both obtaining raw data and interpretation are performed together on a real-time basis.

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Results of NCV reflect on the integrity and function of: 1) the myelin sheath (Schwann cell-derived insulation covering an axon); and, 2) the axon (an extension of the neuronal cell body) of a nerve. Axonal damage or dysfunction generally results in loss of nerve or muscle potential amplitude, whereas demyelination leads to prolongation of conduction time.

The following are examples of appropriate clinical settings where nerve conduction studies are helpful in diagnosing:

- Focal neuropathies or compressive lesions such as carpal tunnel syndrome, ulnar neuropathies or root lesions for localization.
- Traumatic nerve lesions for diagnosis and prognosis.
- Diagnosis or confirmation of suspected generalized neuropathies, such as diabetic, uremic, metabolic, inflammatory or immune.
- Repetitive nerve stimulation in diagnosis of neuromuscular junction disorders such as myasthenia gravis and myasthenic syndromes.

F-wave studies are often performed in conjunction with motor NCS; H-reflex studies involve both sensory and motor nerves and their connections with the spinal cord. The device used must be capable of recording amplitude, duration, response configuration (motor NCV) and latency and sensory nerve action potential amplitudes (sensory NCV).

Electromyography (EMG) is the study of intrinsic electrical properties of skeletal muscle utilizing insertion of a (frequently disposable) needle electrode into muscles of interest. EMG testing relies on both auditory and visual feedback from the electromyographer. EMG results reflect not only the integrity of the functioning connection between a nerve and it's innervated muscle, but on the integrity of the muscle itself. The device used must be capable of recording motor unit recruitment, amplitude, configuration, spontaneous and insertional activity. Use for intraoperative monitoring of central nervous system tissue during the resection of benign and malignant neoplasia and during corrective surgery for scoliosis may also be needed.

The axon innervating a muscle is primarily responsible for the muscles' volitional contraction, survival and trophic functions. Prime examples of diseases characterized by abnormal EMG are disc disease with abnormal nerve compression, amyotrophic lateral sclerosis and neuropathies. Axonal and muscle involvement are most sensitively detected by EMGs, and myelin and axonal involvement are best detected by NCV.

Use of EMG with Botulinum Toxin Injection
EMG may be used to optimize the anatomic location of botulinum toxin injection. It is expected there will be one study performed per anatomic location of injection, if needed. It is expected that the accompanying study to the injection be billed as a limited study (95874) unless supportive documentation is noted to show why more extensive studies are indicated.

Limitations

- Sensory nerve function testing performed with various sensory discrimination and pressure-sensitive devices, including but not limited to current perception testing (e.g., Neurometer®), is not covered. Do not report such testing as nerve conduction testing using any CPT code included in this Policy.

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- Nerve conduction studies and EMG will not be covered if provided in the beneficiary's home.

Providers shall consider a service to be reasonable and necessary if the provider determines that the service is:

- Safe and effective.
- Not experimental or investigational (exception: routine costs of qualifying clinical trial services with dates of service on or after September 19, 2000, which meet the requirements of the clinical trials NCD are considered reasonable and necessary).
- Appropriate, including the duration and frequency that is considered appropriate for the service, in terms of whether it is:
    - Furnished in accordance with accepted standards of medical practice for the diagnosis or treatment of the patient's condition or to improve the function of a malformed body member.
    - Furnished in a setting appropriate to the patient's medical needs and condition.
    - Ordered and furnished by qualified personnel.
    - The EMG must always be ordered, performed and interpreted by a physician trained in electrodiagnostic medicine.
    - The NCS may be performed by a physician or a trained allied health professional working under the direct supervision of a physician trained in electrodiagnostic medicine. The American Association of Neuromuscular and Electrodiagnostic Medicine (AANEM) states, "NCSs should be either (a) performed directly by physician or (b) performed by a trained individual under the direct supervision of a physician. Direct supervision means that the physician is in close physical proximity to the EDX laboratory while testing is underway, is immediately available to provide the trained individual with assistance and direction and is responsible for selecting the appropriate NCSs to be performed". One that meets, but does not exceed, the patient's medical need.
    - At least as beneficial as an existing and available medically appropriate alternative.

Documentation Requirements

Documentation supporting the medical necessity should be legible, maintained in the patient's medical record and made available to Medicaid upon request.

It is expected that the (Nerve Conduction Velocity) NCV and EMG reports will contain data from the study as well as the interpretation and diagnosis.

- In the event of a review for medical necessity, the patient's medical record must support the need for the studies performed. The number of limbs or areas tested should be the minimum needed to evaluate the patient's condition. Repeat testing should be infrequent; limitation of testing services will be determined on the basis of individual medical necessity.
- Documentation addressing the need to evaluate the patient for peripheral neuropathy must be maintained by the practitioner and available upon request.

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- Documentation addressing the indications and circumstances requiring individual nerve conduction studies (without accompanying EMG) must be maintained by the practitioner and made available upon request.
- Credentials of providers billing for needle electromyography must be made available on request. According to the AANEM American Association of Neuromuscular & Electrodiagnostic Medicine, the EMG must be performed and interpreted by a physician who received training during residency and/or in special EDX fellowships after residency. Knowledge of EDX medicine is necessary to pass the board exams given by the American Board of Physical Medicine and Rehabilitation and the American Board of Psychiatry and Neurology.
- The NCS may be performed by a physician or by a trained allied health professional under direct supervision of a physician trained in electrodiagnostic medicine; although always interpreted by a credentialed physician.
- The record must reflect the need for EMG to localize the optimal injection site for the botulinum toxin.

Medicaid would not expect to see multiple uses of EMG in the same patient at the same location for the purpose of optimizing botulinum toxin injections.

Medicaid does not expect to see nerve conduction testing accomplished with discriminatory devices that use fixed anatomic templates and computer-generated reports used as an adjunct to physical examination routinely on all patients.

NOTE:
Medicaid requires the medical necessity for each service reported to be clearly demonstrated in the patient's medical record.

22.3 Prior Authorization (PA) and Referral Radiology Services

A PA is required for all MRI's, MRA's, CT scans, CTA's, and PET scans. Prior Authorization represents a clinical decision regarding medical necessity but is not a guarantee of payment.

The PA requirements will apply to Medicaid recipients who have **full** Medicaid eligibility as defined in Chapter 3, Verifying Recipient Eligibility.

Inpatient hospital, emergency room, Plan First, SOBRA Adult, and Medicaid/Medicare recipients are exempt from the PA requirement.

Send PA requests for outpatient diagnostic imaging procedures to EviCore by:

- Phone at (855) 774-1318
- Fax at (888) 693-3210
- Website at www.evicore.com.

(Normal business hours for phone or fax are 7:00 a.m. to 8:00 p.m. central time)

Only the referring provider and the performing provider may request a PA from EviCore.

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Medicaid does not accept PAs from third party insurers. If the recipient has third party insurance, the provider must obtain a PA from Medicaid.

A provider must obtain a PA before performing a service. The requesting and performing providers will receive notification of the status for the requested PA. If additional information is required, the provider will have fifteen (15) calendar days from the original request date to provide this information. If no information is provided, the PA will be denied.

In the event of urgent care (services furnished within 12 hours to avoid the likely onset of an emergency medical condition or performed after normal work hours (7am – 7pm)) and the PA cannot be obtained prior to performing the test, the provider has thirty (30) calendar days from the date the test was performed to submit a request. EviCore will first determine if it is performed on an urgent basis. If it is determined that the procedure was not perform on an urgent basis, it will deny the PA request. If it is determined to be urgent, it will then go through the normal process to determine medical necessity established by evidenced based clinical guidelines. This criterion is necessary for consideration of a PA. Approved PAs will receive a valid authorization number. **The authorization number will be valid for sixty days from the approval date, not the date of the request.**

***Form 471 is not for reconsiderations of denied PAs or for procedure code changes**. **If changing a procedure code, void the approved PA and submit a new PA request**.

NOTE:

It is the responsibility of all providers to verify the approval of the PA before performing the service.

"Retroactive Eligibility" means applicants may be awarded coverage for a period of time prior to the application approval. Alabama Medicaid sends the “Notification of Retroactive Eligibility” directly to the recipient; they are not sent to the provider. Therefore, the provider must verify eligibility at each visitation.

In the event of retroactive eligibility, the retro date must cover the date of service. Providers have ninety (90) calendar days from the retro issue date to submit a written request indicating the recipient has been awarded retro eligibility and an error free red drop ink claim form, with medical documentation to:
Alabama Medicaid
Attn: Medical Services Division
501 Dexter Avenue
Montgomery, Alabama 36103-56240533.

PA is required for the following radiology codes:

PET SCANS
78459 Myocardial -metabolic
78491 Myocardial-single-rest/stress
78492 Myocardial, perfusion-multi
78608 Brain-metabolic
78811 Limited area

Deleted: ~~78609 Brain, perfusion~~

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78812 Skull base to mid-thigh
78813 Whole body
78814 w/CT; limited area
78815 w/CT skull base to mid-thigh
78816 w/CT full body
CTA
70496 Head
70498 Neck
71275 Chest (non-coronary)
73206 Upper extremity
73706 Lower extremity
75635 CT Angio Abdominal Arteries
CT
70450 Head/brain w/o contrast
70460 Head/brain w/ contrast
70470 Head/brain w/o & w/contrast
70471 CT Scan of Blood Vessels in the Head and Neck with Contrast
70473 CT Scan of Brain Blood Flow with Contrast
70480 Orbit w/o contrast
70481 Orbit w/ contrast
70482 Orbit w/o & w/contrast
70486 Maxllfcl w/o contrast
70487 Maxllfcl w/ contrast
70488 Maxllfcl w/o & w/contrast
70490 Soft tissue neck w/o contrast
70491 Soft tissue neck w/o, w/contrast
70492 Soft tissue neck w/o & w/contrast
71250 Thorax w/o contrast
71260 Thorax w/contrast
71270 Thorax w/o & w/contrast
72125 C-spine w/o contrast
72126 C-spine w/contrast
72127 C-spine w/o & w/contrast
72128 T-spine w/o contrast
72129 T- spine w/contrast
72130 T-spine w/ & w/o contrast
72131 L-spine w/o contrast
72132 L-spine w/contrast
72133 L-spine w/o & w/ contrast
72192 Pelvis w/o contrast

Added: 70471 CT Scan…Flow with Contrast.

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72191 CT Angiograph Pelv W/O&W/Dye
72193 Pelvis w/contrast
72194 Pelvis w/o & w/ contrast
73200 UE- w/o contrast
73201 UE- w/contrast
73202 UE w/o & with contrast
73700 LE w/o contrast
73701 LE w/ contrast
73702 LE w/o & w/contrast
74150 Abdomen w/o contrast
74160 Abdomen w/contrast
74170 Abdome w/o & w/contrast
74174 CT Angio Abd & Pelv W/O&W/Dye
74176 Abdomen & Pelvis w/o contrast
74177 Abdomen & Pelvis with contrast
74178 Abdomen & Pelvis; w/o & w/contrast
75571 Heart w/o contrast.
75572 Heart with contrast.
75573 Heart with contrast, in the setting of CHD.
75574 Heart CT Angiography.
76380 Limited or localized f/u study
77078 CT Bone Density Axial
MRA
70544 Head w/o contrast
70545 Head w/contrast
70546 Head w/ & w/o contrast
70547 Neck w/o contrast
70548 Neck w/contrast
70549 Neck w/o & w/contrast
71555 Chest w/ or w/o contrast
72198 Pelvis w/ or w/o contrast
73225 UE w/ or w/o contrast
73725 LE w/ or w/o contrast
74174 CTA, Abdomen and pelvis with contrast
74185 Abdomen w/ or w/o contrast
MRI
70336 Scan of Jaw Joint
70540 Face, orbit, &/or neck w/o contrast
70542 Face orbit &/or neck w & w/o cont.
70543 Face, orbit, &/or neck w & w/o cont.

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70551 Brain w/o contrast
70552 Brain w/ contrast
70553 Brain w/& w/o contrast
71550 Chest w/o contrast
71551 Chest w/ contrast
71552 Chest w & w/o contrast
72141 C-spine w/o contrast
72142 C-spine w/contrast
72146 T-spine w/o contrast
72147 T-spine w/contrast
72148 L-spine w/o contrast
72149 L spine w/contrast
72156 c-spine w/ & w/o contrast
72157 T-spine w/ & w/o contrast
72158 L-spine w/ & w/o contrast
72195 Pelvis w/o contrast
72196 Pelvis w/ contrast
72197 Pelvis w/&w/o contrast
73218 UE w/o contrast
73219 UE w/contrast
73220 UE w/& w/o contrast
73221 UE joint w/o contrast
73222 UE joint w/contrast
73223 UE joint w/& w/o contrast
73718 LE w/o contrast
73719 LE w/ contrast
73720 LE w/ & w/o contrast
73721 LE joint w/o contrast
73722 LE joint w/ contrast
73723 LE joint w & w/o contrast
74181 Abdomen w/o contrast
74182 Abdomen w/contrast
74183 Abdomen w & w/o contrast
75557 Cardiac w/o contrast
77046 MRI of one breast
77047 MRI of both breast
77048 MRI of one breast with and without contrast
77049 MRI of both breast with and without contrast

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22.4 Prior Authorization (PA) and Referral Requirements Cardiology Services

A PA is required for all Nuclear Cardiology, Diagnostic Heart catheterization, Stress Test (ECHO). A prior authorization represents a clinical decision regarding medical necessity but is not a guarantee of payment.

The PA requirements will apply to Medicaid recipients who have full Medicaid eligibility as defined in Chapter 3 Verifying Recipient Eligibility.

Inpatient hospital, emergency room, Plan First, SOBRA Adults, and Medicaid/ Medicare recipients are exempt from the PA requirement.

Send PA requests for outpatient diagnostic imaging procedures to EviCore by:

- Phone at (855) 774-1318
- Fax at (888) 693-3210
- Website at www.evicore.com.

(Normal business hours for phone or fax are 7:00am to 8:00pm central time)

Only the referring provider and the performing provider may request a PA from EviCore.

Medicaid does not accept PAs from third party insurers. If a recipient has third party insurance, the provider must obtain a PA from Medicaid.

A provider must obtain a PA before performing a test. The requesting and performing providers will receive notifications of the status for the requested PA. If additional information is required, the provider will have fifteen (15) calendar days from the original request date to provide this information. If no information is provided, the PA will be denied.

In the event of urgent care (services furnished within 12 hours to avoid the likely onset of an emergency medical condition or performed after normal working hours (7am – 7pm)) and the PA cannot be obtained prior to performing the test, the provider has thirty (30) calendar days to submit a request from the date the test was performed. EviCore will first determine if it is performed on an urgent basis. If it is determined that the procedure was not perform on an urgent basis, it will deny the PA request. If it is determined to be urgent, it will then go through the normal process to determine medical necessity established by evidence based clinical guidelines. This criterion is necessary for consideration of a PA. Approved PAs will receive a valid authorization number. The authorization number will be valid for sixty days from the approval date, not the date of the request.

*Form 471 form is not for reconsiderations of denied PAs or for procedure code changes. If changing a procedure code, void the approved PA and submit a new PA request.

NOTE:

It is the responsibility of all providers to verify the approval of the PA before performing the service.

"Retroactive Eligibility" means applicants may be awarded coverage for a period of time prior to the application approval. Alabama Medicaid sends the “Notification of Retroactive Eligibility” directly to the recipient; they are not sent to the provider. Therefore, the provider must verify eligibility at each visitation.

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In the event of retroactive eligibility, the retro date must cover the date of service. Providers have ninety (90) calendar days from the retro issue date to submit a written request indicating the recipient has been awarded retro eligibility, and an error free drop ink claim form with medical documentation to:

Alabama Medicaid
Attn: Medical Services Division
501 Dexter Avenue
Montgomery, Alabama 36103-5624

PA is required for the following cardiology codes:

HEART CATHERIZATION
93451 RIGHT HEART CATHETERIZATION INCLUDING MEASUREMENT(S) OF OXYGEN SATURATION AND CARDIAC OUTPUT, WHEN PERFORMED.
93452 LEFT HEART CATHETERIZATION INCLUDING INTRAPROCEDURAL INJECTION(S) FOR LEFT VENTRICULOGRAPHY, IMAGING SUPERVISION AND INTERPRETATION, WHEN PERFORMED
93453 COMBINED RIGHT AND LEFT HEART CATHETERIZATION INCLUDING INTRAPROCEDURAL INJECTION(S) FOR LEFT VENTRICULOGRAPHY, IMAGING SUPERVISION AND INTERPRETATION, WHEN PERFORMED
93454 CATHETER PLACEMENT IN CORONARY ARTERY(S) FOR CORONARY ANGIOGRAPHY, INCLUDING INTRAPROCEDURAL INJECTION(S) FOR CORONARY ANGIOGRAPHY, IMAGING SUPERVISION AND INTERPRETATION;
93455 CATHETER PLACEMENT IN CORONARY ARTERY(S) FOR CORONARY ANGIOGRAPHY, INCLUDING INTRAPROCEDURAL INJECTION(S) FOR CORONARY ANGIOGRAPHY, IMAGING SUPERVISION AND INTERPRETATION; WITH CATHETER PLACEMENT(S) IN BYPASS GRAFT(S) (INTERNAL MAMMARY, FREE ARTERIAL, VENOUS GRAFTS) INCLUDING INTRAPROCEDURAL INJECTION(S) FOR BYPASS GRAFTANGIOGRAPHY
93456 CATHETER PLACEMENT IN CORONARY ARTERY(S) FOR CORONARY ANGIOGRAPHY, INCLUDING INTRAPROCEDURAL INJECTION(S) FOR CORONARY ANGIOGRAPHY, IMAGING SUPERVISION AND INTERPRETATION; WITH RIGHT HEART CATHETERIZATION
93457 CATHETER PLACEMENT IN CORONARY ARTERY(S) FOR CORONARY ANGIOGRAPHY, INCLUDING INTRAPROCEDURAL INJECTION(S) FOR CORONARY ANGIOGRAPHY, IMAGING SUPERVISION AND INTERPRETATION; WITH CATHETER PLACEMENT(S) IN BYPASS GRAFT(S) (INTERNAL MAMMARY, FREE ARTERIAL, VENOUS GRAFTS) INCLUDING INTRAPROCEDURAL INJECTION(S) FOR BYPASS GRAFT ANGIOGRAPHY AND RIGHT HEART CATHETERIZATION
93458 CATHETER PLACEMENT IN CORONARY ARTERY(S) FOR CORONARY ANGIOGRAPHY, INCLUDING INTRAPROCEDURAL INJECTION(S) FOR CORONARY ANGIOGRAPHY, IMAGING SUPERVISION AND INTERPRETATION; WITH LEFT HEART CATHETERIZATION INCLUDING INTRAPROCEDURAL INJECTION(S)
93459 CATHETER PLACEMENT IN CORONARY ARTERY(S) FOR CORONARY ANGIOGRAPHY, INCLUDING INTRAPROCEDURAL INJECTION(S) FOR CORONARY ANGIOGRAPHY, IMAGING SUPERVISION AND INTERPRETATION; WITH LEFT HEART CATHETERIZATION INCLUDING INTRAPROCEDURAL INJECTION(S) FOR LEFT VENTRICULOGRAPHY, WHEN PERFORMED, CATHETER PLACEMENT(S) IN

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HEART CATHERIZATION
BYPASS GRAFT(S) (INTERNAL MAMMARY, FREE ARTERIAL, VENOUS GRAFTS) WITH BYPASS GRAFT ANGIOGRAPHY
93460 CATHETER PLACEMENT IN CORONARY ARTERY(S) FOR CORONARY ANGIOGRAPHY, INCLUDING INTRAPROCEDURAL INJECTION(S) FOR CORONARY ANGIOGRAPHY, IMAGING SUPERVISION AND INTERPRETATION; WITH RIGHT AND LEFT HEART CATHETERIZATION INCLUDING INTRAPROCEDURAL INJECTION(S) FOR LEFT VENTRICULOGRAPHY, WHEN PERFORMED
93461 CATHETER PLACEMENT IN CORONARY ARTERY(S) FOR CORONARY ANGIOGRAPHY, INCLUDING INTRAPROCEDURAL INJECTION(S) FOR CORONARY ANGIOGRAPHY, IMAGING SUPERVISION AND INTERPRETATION; WITH RIGHT AND LEFT HEART CATHETERIZATION INCLUDING INTRAPROCEDURAL INJECTION(S) FOR LEFT VENTRICULOGRAPHY, WHEN PERFORMED, CATHETER PLACEMENT(S) IN BYPASS GRAFT(S) (INTERNAL MAMMARY, FREE ARTERIAL, VENOUS GRAFTS) WITH BYPASS GRAFT ANGIOGRAPHY
NUCLEAR CARDIOLOGY
78451 MYOCARDIAL PERFUSION IMAGING, TOMOGRAPHIC (SPECT) (INCLUDING ATTENUATION CORRECTION, QUALITATIVE OR QUANTITATIVE WALL MOTION, EJECTION FRACTION BY FIRST PASS OR GATED TECHNIQUE, ADDITIONAL QUANTIFICATION, WHEN PERFORMED); SINGLE STUDY, AT REST OR STRESS (EXERCISE OR PHARMACOLOGIC)
78452 MYOCARDIAL PERFUSION IMAGING, TOMOGRAPHIC (SPECT) (INCLUDING ATTENUATION CORRECTION, QUALITATIVE OR QUANTITATIVE WALL MOTION, EJECTION FRACTION BY FIRST PASS OR GATED TECHNIQUE, ADDITIONAL QUANTIFICATION, WHEN PERFORMED); MULTIPLE STUDIES, AT REST AND/OR STRESS (EXERCISE OR PHARMACOLOGIC) AND/OR REDISTRIBUTION AND/OR REST REINJECTION
78453 MYOCARDIAL PERFUSION IMAGING, PLANAR (INCLUDING QUALITATIVE OR QUANTITATIVE WALL MOTION, EJECTION FRACTION BY FIRST PASS OR GATED TECHNIQUE, ADDITIONAL QUANTIFICATION, WHEN PERFORMED); SINGLE STUDY, AT REST OR STRESS (EXERCISE OR PHARMACOLOGIC)
78454 MYOCARDIAL PERFUSION IMAGING, PLANAR (INCLUDING QUALITATIVE OR QUANTITATIVE WALL MOTION, EJECTION FRACTION BY FIRST PASS OR GATED TECHNIQUE, ADDITIONAL QUANTIFICATION, WHEN PERFORMED); MULTIPLE STUDIES, AT REST AND/OR STRESS (EXERCISE OR PHARMACOLOGIC) AND/OR REDISTRIBUTION AND/OR REST REINJECTION
STRESS ECHO
93350 ECHOCARDIOGRAPHY, TRANSTHORACIC, REAL-TIME WITH IMAGE DOCUMENTATION (2D), INCLUDES M-MODE RECORDING, WHEN PERFORMED, DURING REST AND CARDIOVASCULAR STRESS TEST USING TREADMILL, BICYCLE EXERCISE AND/OR PHARMACOLOGICALLY INDUCED STRESS, WITH INTERPRETATION AND REPORT;
93351 ULTRASOUND EXAMINATION AND CONTINUOUS MONITORING OF THE HEART PERFORMED DURING REST, EXERCISE, AND/OR DRUG-INDUCED STRESS WITH INTERPRETATION AND REPORT 93352 INJECTION OF X-RAY CONTRAST MATERIAL FOR ULTRASOUND EXAMINATION OF THE HEART

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22.5 Provider Rights

Prior authorizations are required for certain radiology and cardiology codes. The Alabama Medicaid Agency contracts with EviCore to review providers' prior authorization requests of diagnostic radiology and cardiology services authorized under the provisions of the Alabama Medicaid Administrative Code Rule 560-X-34.07 and Chapter 22 of the Medicaid Provider Manual.

Providers have the right to accept or disagree with the prior authorization decision for coverage of medically necessary services. EviCore provides an online manual of evidence-based criteria for radiology and cardiology procedures covered by Medicaid. Providers may also request a written statement of the clinical standards used in making a determination by contacting EviCore at the following address:

EviCore Healthcare
Provider Response Unit
730 Cool Springs Blvd., Suite 800
Franklin, TN 37067

Once the prior authorization has been determined, the requesting provider (or their staff) must advise each recipient when a prior authorization is denied prior to services being rendered and advise each recipient of their responsibility for any billed charges. The recipient's decision should then be recorded in the medical record.

If the decision by the provider is to request a review of the initial prior authorization denial, the following section describes the review process by EviCore and Alabama Medicaid Agency towards providers participating in the Alabama Medicaid Program.

Radiology/Cardiology Prior Authorization Denial Appeal Process

Step 1: Reconsideration or Peer to Peer Review
The requesting provider may contact EviCore's Medical Management Department at (855) 774-1318 to discuss this case. EviCore must receive a request by one of following two options within **30 calendar days** of this letter:

- **Peer to Peer (P2P):** The requesting physician may verbally request to speak with EviCore's Medical Director.
- **Reconsideration:** The requesting physician may submit a formal written request to speak with EviCore's Medical Director.

The Reconsideration and the P2P is an opportunity for the provider to discuss the case and the medical documentation sent for review.

Step 2: Appeal

If you are unhappy with the Peer to Peer or Reconsideration decision, the requesting physician may request an **appeal** by contacting EviCore's Medical Director at 1-800-918-8924, ext.11758, also within **30 calendar days** of the initial denial letter. Please use the reference code number when submitting additional medical documents, records, and other information relevant to the request.

The request for an appeal must include additional medical documentation.

*EviCore must receive requests for appeal along with relevant documents within 30 days from the date of the denial letter.

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NOTE: Prior Authorization request, P2P, Reconsideration, and Appeals
Standard prior authorization request will be processed within seven (7) calendar days, and expedited prior authorization request will be processed within 72 hours after receipt of the request.

Step 3: Fair Hearing

A fair hearing is a face-to-face hearing by an impartial State Hearing Officer at a time and place reasonably convenient for the complainant and attended by the complainant or his authorized representatives who may call witnesses or examine witnesses called by others.

If the provider is unsatisfied with the Reconsideration decision, the provider has the right to request a Fair Hearing. The request for a Fair Hearing must include medical documentation not previously presented in the original request, such as:

- Prior diagnostic studies results
- Medication changes (including dose and duration)
- Clinical notes (i.e.: current signs & symptoms, prior medical management, etc.)
- Operative notes

*Please use the reference code number when submitting additional documentation.

Whether or not a reconsideration, P2P or appeal is requested, a fair hearing request must be received by the Office of General Counsel within 60 days of the date of the initial denial letter. Medicaid will not accept requests for Fair Hearings or Undue Hardship, if allowed by law, which are outside the 60-day limit.

The request must identify all new or supplemental documentation. Send written requests for a Fair Hearing to:

Alabama Medicaid Agency
Attn: Office of General Counsel
501 Dexter Avenue
P. O. Box 5624
Montgomery, AL 36103-5624

Once your hearing request is complete, you will receive a letter from a hearing officer providing information about hearing procedures and setting the time and place for the hearing. **If you are satisfied before the hearing and want to withdraw your request, you or your representative should give written notice to the Medicaid Agency.**

22.6 Cost Sharing (Copayment)

Copayment amount does not apply to services provided by Independent Radiology providers.

22.7 Completing the Claim Form

To enhance the effectiveness and efficiency of Medicaid processing, provider should bill Medicaid claims electronically.

Radiology providers who bill Medicaid claims electronically receive the following benefits:

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- Quicker claim processing turnaround
- Immediate claim correction
- Enhanced online adjustment functions
- Improved access to eligibility information

Refer to Appendix B, Electronic Media Claims Guidelines, for more information about electronic filing.

NOTE:

When filing a paper claim, a CMS-1500 claim form is required. Medicare-related claims must be filed using the Medical Medicaid/Medicare-related Claim Form 340. All paper claims must be a one page error free red drop ink form.

This section describes program-specific claims information. Refer to Chapter 5, Filing Claims, for general claims filing information and instructions.

22.7.1 Time Limit for Filing Claims

Medicaid requires all claims for Independent Radiology providers to be filed within one year of the date of service. Refer to Chapter 5, Filing Claims, for more information regarding timely filing limits and exceptions.

22.7.2 Diagnosis Codes

For dates of service 01/01/99 and after valid diagnosis codes are required. The International Classification of Diseases - 10th Revision - Clinical Modification (ICD-10-CM) manual lists Medicaid required diagnosis codes. These manuals may be obtained by contacting the American Medical Association, AMA Plaza 330 North Wabash Ave, Suite 39300 Chicago, IL 60611-5885, or 1-800-621-8335.

For dates of service prior to 01/01/99, Independent Radiology providers are not required to provide valid diagnosis codes. Providers must bill diagnosis ICD 10 Z01.89 on hard copy and electronically submitted claims.

NOTE:

ICD-10 codes should be used for claims submitted with dates of service on/after 10/01/2015.

NOTE:

ICD-10 diagnosis codes must be listed to the highest number of digits possible (3, 4, or 5 digits). Do not use decimal points in the diagnosis code field.

22.7.3 Procedure Codes and Modifiers

Radiology providers use the Current Procedural Terminology (CPT) coding system. The CPT manual lists most required procedure codes. This manual may be obtained by contacting the Order Department, American Medical Association, P. O. Box 930876 Atlanta, GA 31193-0873 or 1-800-621-8335.

Radiology Facilities are limited to billing CPT radiology procedure codes. The range of codes is 70010 through 79999. Physiological labs are restricted to the codes listed in their contract with Medicaid.

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The (837) Professional, Institutional and Dental electronic claims and the paper claim have been modified to accept up to four Procedure Code Modifiers.

Professional and Technical Components

Some procedure codes in the 70000 and 90000, and G series are a combination of a professional component and a technical component. Therefore, these codes may be billed three different ways; (1) as a global, (2) as a professional component, or (3) as a technical component.

- **Global**, the provider must own the equipment, pay the technician, review the results, and provide a written report of the findings. The procedure code is billed with no modifiers.
- **Professional component**, the provider does not own the equipment. The provider operates the equipment and/or reviews the results, and provides a written report of the findings. The Radiological professional component is billed by adding modifier 26 to the procedure code.
- **Technical component**, the provider must own the equipment but does not review and document the results. The technical component is billed by adding modifier TC to the procedure code.

22.7.4 Place of Service Codes

Radiology service claims may be filed using, but not limited to, the following place of service:

POS Code	*Description*
11	Clinic

22.7.5 Required Attachments

To enhance the effectiveness and efficiency of Medicaid processing, your attachments should be limited to claims with third party denials.

NOTE:
When an attachment is required, submit a hard copy error free red drop ink form.

Refer to Chapter 5, for more information on required attachments.

22.8 For More Information

This section contains a cross-reference to other relevant sections in the manual.

Resource	Where to Find It
CMS-1500 Claim Filing Instructions	Chapter 5
Medical Medicaid/Medicare-related Claim Filing Instructions	Chapter 5
Medical Necessity/Medically Necessary Care	Chapter 7
Electronic Media Claims (EMC) Submission Guidelines	Appendix B
AVRS Quick Reference Guide	Appendix L
Alabama Medicaid Contact Information	Appendix N
Medicaid Forms	Appendix E

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